NCCP 00225 · VERSION 6

Your treatment, explained

Advanced / metastatic colorectal cancer · NCCP 00225 v6 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Panitumumab

14-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: hse · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Treatment, review and the next cycle
- Your 14-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Your treatment aim and review plan
- Before you start and before each cycle
- Living with treatment
- When treatment changes or finishes
- Recognise changes early
- Visits, access and coordination
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

How your treatment works

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

EGFR-TARGETED ANTIBODY

Panitumumab

Vectibix



Blocks growth messages at the cell surface

Panitumumab attaches to EGFR, a receptor on the outside of cells. This can interrupt signals that help some bowel cancers grow. A tumour RAS test helps identify whether this approach is suitable. EGFR also supports healthy skin, which explains why skin and nail changes need attention.

EGFR receives a growth signal → Panitumumab blocks the receptor → Some growth signals are reduced

What to watch for

Report rash, painful cracks or nail changes early. Eye pain or blurred vision, new breathing symptoms, severe diarrhoea, or blistering skin need prompt assessment. Blood tests check minerals such as magnesium.

Sources: smpc · leaflet

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Skin / rash

Panitumumab

Report rash, cracks, painful or infected skin early.

3 Breathing

Panitumumab

New or worsening breathlessness or cough needs assessment.

5 Nails

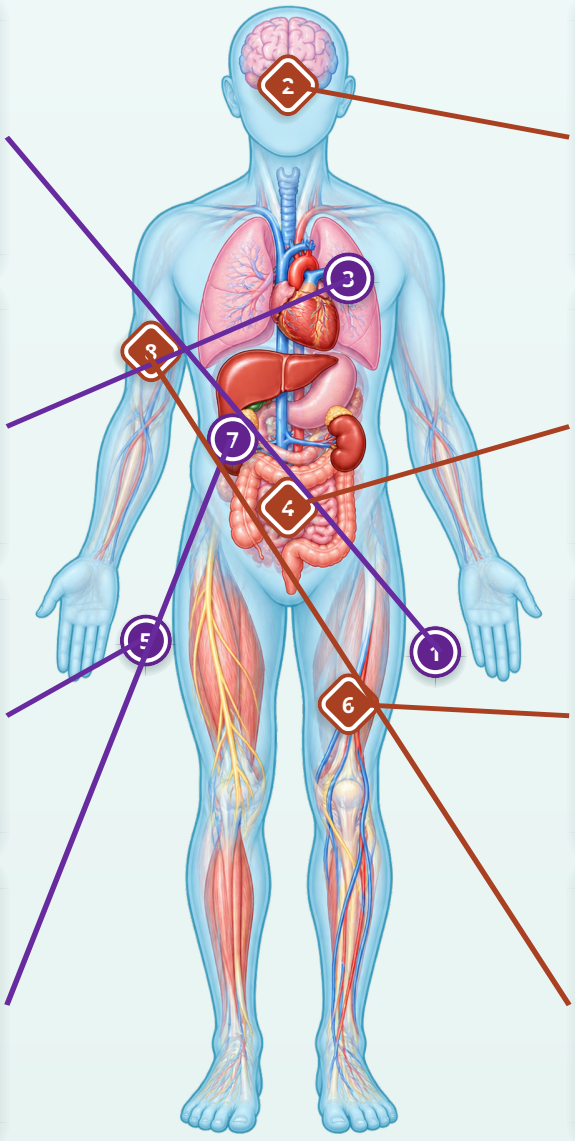
Panitumumab

Painful or infected nail folds need early treatment.

7 Hydration

Panitumumab

Reduced urine or severe dehydration needs urgent advice.



2 Eyes

Panitumumab

New eye pain, light sensitivity or blurred vision: prompt advice.

4 Bowel

Panitumumab

Severe diarrhoea, dizziness or inability to drink: urgent contact.

6 Blood minerals

Panitumumab

Tests check magnesium, calcium and potassium; report cramps or weakness.

8 Infusion reaction

Panitumumab

Tell the nurse about swelling, itching, flushing or breathing changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Treatment, review and the next cycle

INFUSION → HOME → REVIEW

A repeating 14-day cycle



Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

Check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver	Baseline blood count, kidney and liver profile	Review results before treatment	Blood count, kidney and liver tests before each treatment	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Magnesium, calcium and potassium	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Cancer response and treatment choices	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00225 v6 · official regimen · Panitumumab product information

THE WHOLE TREATMENT JOURNEY

Panitumumab: treatment and checks

Advanced / metastatic colorectal cancer

Panitumumab on Day 1 every 14 days while beneficial and tolerable. There is no home chemotherapy pump.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver NCCP 00225	Baseline blood count, kidney and liver profile	Review results before treatment	Blood count, kidney and liver tests before each treatment	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Magnesium, calcium and potassium NCCP 00225	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing NCCP 00225	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Cancer response and treatment choices NCCP 00225	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

What each check is for

1

A repeating 14-day cycle

Infusion → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00225

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.34

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Panitumumab on Day 1 every 14 days while beneficial and tolerable. There is no home chemotherapy pump.

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14
Panitumumab infusion	IV													
Skin / nails	May develop or accumulate over cycles													
Bowel / fatigue	Timing varies													
Mineral changes	Blood tests during and after treatment													

The planned treatment: Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Blood tests after the final dose: Mineral checks continue for about eight weeks after panitumumab. There is no guaranteed safe day for symptoms.

Skin care continues between visits: Rash or nail changes may build over several cycles. Report symptoms early and follow your written plan.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00225 v6 · official regimen ↗️ · Panitumumab product information ↗️

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Read the whole cycle as text

Day 1: Review and infusion

The team checks symptoms, blood tests and treatment suitability, then gives panitumumab through a vein. Tell the nurse immediately about infusion symptoms. Confirm the skin-care plan, contact number and next visit.

Day 2–7: At home

There is no chemotherapy pump to disconnect. Watch skin, bowel and eye symptoms and use the prescribed care plan. Report changes early; serious symptoms can happen on any day.

Day 8–14: Continue care and prepare for review

Rash, nail changes and mineral problems may build over several cycles. Feeling well does not replace blood tests. Bring your symptom record. Do not wait for the next infusion if symptoms are concerning.

PRACTICAL TREATMENT GUIDE

Visits, access and care between treatments

Panitumumab on Day 1 every 14 days while beneficial and tolerable. There is no home chemotherapy pump.

STEP 1

Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Ask whether a peripheral cannula or an existing line is appropriate; a port is not automatically required.

STEP 2

At the treatment unit

The first panitumumab infusion usually takes about one hour; higher doses or reactions may need longer. Later tolerated infusions may be shorter. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.

STEP 3

After the infusion

There is no fluorouracil home pump and no disconnection visit in this regimen.

STEP 4

Between doses

Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00225 v6 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Start skin care early

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice.

2 Minerals and blood tests

Panitumumab can lower magnesium and other minerals even when you feel well. Blood tests continue during treatment and for about eight weeks after it finishes. Take replacement treatment only as prescribed.

3 Feeling sick or loose stools

Panitumumab has low nausea risk, but symptoms can still occur. Follow the rescue medicine and hydration plan from your team. Severe or persistent diarrhoea, reduced urine or inability to drink needs urgent advice.

4 Travel and daily life

Allow time for blood tests, review, preparation, infusion and observation. Ask about transport if medicines or symptoms affect driving. Pace activity and seek help with food, work or caring responsibilities.

5 The whole treatment plan

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

Sources: nccp · smpc · hse

TREATMENT REFERENCE · 01

Your treatment aim and review plan

Understand why this option fits your cancer.

The setting matters

Panitumumab is used for bowel cancer that has spread and has a confirmed wild-type RAS result. It is given after earlier fluoropyrimidine, oxaliplatin and irinotecan treatment has failed. Your team explains the aim, alternatives and how benefit will be reviewed.

Sources: nccp

What the tumour tests mean

RAS wild type is required for panitumumab. Your team also considers the cancer location, other molecular results, previous treatment and whether surgery or other local treatment could be useful. A positive eligibility result is not a promise of benefit.

Sources: nccp · smpc

TREATMENT REFERENCE · 02

Before you start and before each cycle

The team checks that the plan remains suitable.

Checks and medicines

Review blood counts, kidney/liver function and minerals, current symptoms, allergies and all medicines. DPD testing is not a requirement for panitumumab alone.

Sources: nccp · smpc

Skin, eyes and breathing

Tell the team about an existing rash, skin infection, dry or painful eyes, contact lenses, or previous lung inflammation. Report new eye pain, light sensitivity or visual change promptly.

Sources: smpc · leaflet

TREATMENT REFERENCE · 03

Living with treatment

Build the plan around what matters to you.

Skin and nails

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice. Avoid friction on sore skin and tell the team about painful nail folds.

Sources: smpc · leaflet

Food, activity and support

Keep fluids and nourishing food within reach. Pace activity around energy levels and ask for dietetic, emotional or practical support. If symptoms prevent eating, drinking or usual tasks, contact the team.

Sources: hse

Pregnancy and fertility

Discuss pregnancy plans and fertility before treatment. Panitumumab requires contraception during treatment and for two months afterwards, and breastfeeding should be avoided for that period. When chemotherapy is also included, its precautions may last longer; ask for the instructions for the whole regimen.

Sources: smpc

TREATMENT REFERENCE · 04

When treatment changes or finishes

Follow-up continues after the last infusion.

Monitoring and recovery

Continue your skin plan and planned magnesium, calcium and potassium checks for about eight weeks after panitumumab. New symptoms still need review. Ask when to stop prescribed supportive medicines.

Sources: nccp · smpc

Treatment choices

Your team may pause, reduce, change or stop one or more medicines. Ask which medicine changes, why, and what the next review will assess. Supportive and palliative care can help alongside active treatment.

Sources: nccp

TREATMENT REFERENCE · 05

Recognise changes early

You do not need to identify the responsible medicine before asking for help.

When to call

Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly.

Sources: smpc · hse

Track impact, not just a score

Describe when symptoms began, whether they are worsening, and whether they affect food, fluids, sleep, walking or daily tasks. Your team assigns the clinical grade. Use the passport if it helps; it is not required to access this information.

Sources: hse

Visits, access and coordination

Know what happens in the clinic and what happens at home.

The delivery plan

Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Sources: nccp

Radiotherapy or other treatment

This regimen has no routine radiotherapy component. If another treatment is proposed, ask whether it happens before, during or after this course and who will coordinate the appointments.

Sources: nccp

20100007: panitumumab monotherapy

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

Panitumumab improved survival in a trial group selected using tumour testing after other bowel-cancer medicines had stopped working. The likely benefit for you depends on your cancer, previous treatment and general health; it cannot be predicted from the trial alone.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Why do I need a RAS test?

RAS is part of the growth-signalling pathway inside cancer cells. If it is changed by a mutation, blocking EGFR is unlikely to help. Your team needs a validated wild-type RAS result before panitumumab. Other tumour tests and the treatment aim also affect the choice.

Is this the same as checkpoint immunotherapy?

Panitumumab is a targeted antibody against EGFR. It works differently from checkpoint treatments used in some other cancer regimens. Ask your team which treatment class and side effects apply to you.

Does a rash mean the treatment is working?

A rash is a recognised drug effect, but it cannot tell you whether your cancer is responding. Report it early so it can be treated. Scans, symptoms and clinical review guide the response assessment.

Will I need radiotherapy as part of this cycle?

These panitumumab regimens do not contain a scheduled radiotherapy course. If radiotherapy is recommended separately, your oncology teams will explain its purpose and timing. Do not assume it occurs on infusion or pump days.

What about pregnancy, breastfeeding or fertility?

Discuss pregnancy plans and fertility before treatment. Panitumumab requires contraception during treatment and for two months afterwards, and breastfeeding should be avoided for that period. When chemotherapy is also included, its precautions may last longer; ask for the instructions for the whole regimen.

How long should I allow for the appointment?

The infusion itself is usually about an hour at first, but blood tests, review, pharmacy preparation and observation add time. Ask your unit for its expected visit length. Larger doses or a previous reaction may need a longer infusion.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00225 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00225 v6 · official regimen
- Panitumumab product information
- Panitumumab patient medicine leaflet
- HSE: chemotherapy side effects and urgent advice
- 20100007 primary report
- NCCP NCIS supportive antiemetic medicines V8 (2025)

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6560/225_Panitumumab_6mgkg.pdf

https://www.medicines.org.uk/emc/product/6178/smpc

https://www.medicines.org.uk/emc/product/6178/pil

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://pubmed.ncbi.nlm.nih.gov/27736842/

https://healthservice.hse.ie/documents/7180/
Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf